Response to HHS Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Document Number: 2025-23641 (90 FR 60108)

Submitted by:
Center for Elders' Independence
510 17th Street
Oakland, CA 94612

Date: February 11, 2026

To:
Department of Health and Human Services
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

Re: HHS Health Sector AI RFI

Introduction

On behalf of the Center for Elders' Independence (CEI), I am pleased to submit this response to the Department of Health and Human Services' Request for Information on accelerating the adoption and use of artificial intelligence in clinical care. As the Chief Information Officer for CEI, a non-profit Program of All-Inclusive Care for the Elderly (PACE) organization that has served the East Bay of California for over 40 years, I offer a perspective grounded in the practical realities of deploying technology to support frail, medically complex seniors.

CEI operates under a fully capitated, value-based care model, assuming complete financial and clinical responsibility for our participants. This structure aligns our incentives directly with the goals articulated in this RFI: improving health outcomes, reducing costs, and fostering innovation. We currently serve participants across Alameda and Contra Costa Counties through six PACE centers, providing integrated services spanning medical care, rehabilitation, transportation, nutrition, and social engagement. In 2025 alone, our teams provided over 9,300 doctor visits, served nearly 79,000 meals, and coordinated over 171,000 transportation trips for our participants.

At CEI, we adopted the term **"Augmented Intelligence"** to describe our approach to AI. This framing is intentional. It emphasizes that these technologies are designed to enhance and support the clinical judgment, compassion, and expertise of our care teams and not to replace them. This philosophy has been instrumental in building trust and fostering adoption among our staff, and we believe it represents a model that HHS could promote more broadly.

The following responses address each of the ten specific questions outlined in the RFI, drawing on our experience as both configuring and deploying technology solutions for geriatric care.

Responses to Specific Questions

Question 1: Barriers to Private Sector Innovation and Adoption

The most significant barriers to private sector innovation in AI for healthcare, and its adoption in clinical care settings like ours, fall into four interconnected categories.

Data Fragmentation and Interoperability Challenges. The PACE model is predicated on holistic, coordinated care. Yet the health data for our participants is scattered across a fragmented ecosystem of external specialists, hospitals, pharmacies, and diagnostic laboratories. Despite progress on standards like FHIR, true interoperability remains elusive. This fragmentation makes it extraordinarily difficult to assemble the comprehensive, longitudinal datasets required to train, validate, and deploy effective AI models for applications such as predictive risk stratification or personalized care planning.

Data Quality and the Risk of Algorithmic Bias. The frail elderly population we serve presents complex, multi-morbid health profiles and an ethnically diverse population that are systematically underrepresented in the large datasets used to train most commercial AI models. This creates a substantial risk of algorithmic bias, where tools may perform poorly or produce inequitable outcomes for our participants. Compounding this issue is the fact that much of the richest clinical information resides in unstructured formats, such as clinician notes, care conference summaries, and social worker assessments. These are difficult and costly to process for model development.

High Upfront Costs and Uncertain Return on Investment. As a non-profit organization, CEI must be a responsible steward of its resources. The capital investment required for AI infrastructure, specialized talent, and software licensing is substantial. The return on this investment is often realized not in direct revenue, but in improved quality of life, reduced emergency department visits, and delayed or avoided nursing home placements. Without clear reimbursement pathways or quality incentives tied to AI-driven interventions, building a compelling business case for these investments remains a significant challenge.

Workforce Integration and Change Management. Our clinicians and care coordinators are our most valuable asset. For any technology to succeed, it must integrate seamlessly into their existing workflows and demonstrably reduce their administrative burden. Tools that are perceived as adding complexity, requiring extra steps, or undermining clinical autonomy will face resistance. The "Augmented Intelligence" framing is essential to our change management strategy, as it positions technology as a partner that empowers staff rather than a threat that diminishes their role.

Question 2: Recommended Regulatory, Payment, and Programmatic Changes

HHS should prioritize changes that address the foundational barriers to AI adoption while creating positive incentives for innovation.

Mandate and Incentivize True Interoperability. The single most impactful action HHS can take is to move beyond encouraging interoperability to actively mandating and incentivizing it. This includes strengthening enforcement of existing information blocking rules and creating direct financial incentives for providers and health IT vendors to implement and utilize modern, API-based data exchange standards. Until data can flow freely and securely across the care continuum, the promise of AI in healthcare will remain largely unrealized.

Develop AI-Specific Reimbursement Pathways. The current fee-for-service payment system is ill-suited to reward the preventive, proactive interventions that AI enables. HHS should develop and pilot new reimbursement mechanisms that recognize the value of AI-driven care. For value-based care models like PACE, this could include enhanced capitation rates for organizations that demonstrate effective use of AI for risk prediction and care management, or shared savings programs that reward the avoidance of costly acute care episodes.

Establish Regulatory Clarity and Safe Harbors. Regulatory ambiguity is a significant deterrent to innovation. HHS should provide clear, practical guidance on the regulatory pathway for non-medical device AI tools used in clinical settings. Furthermore, HHS should consider establishing regulatory "sandboxes" or safe harbors that allow value-based care organizations to pilot and validate new AI applications in a controlled environment, with defined parameters for patient safety and data privacy, and a degree of protection from punitive action for good-faith efforts to innovate.

Question 3: Novel Legal and Implementation Issues for Non-Medical Devices

The deployment of non-medical device AI in clinical care introduces several novel legal and implementation challenges that existing governance structures are not fully equipped to address.

Accountability and Liability. When an AI tool provides a recommendation that informs a clinical decision, the traditional chain of accountability becomes blurred. If an adverse outcome occurs, the question of liability and who it rests with, be that the software developer, the healthcare organization that deployed the tool, or the individual clinician who acted on the recommendation, is often unclear. HHS can play a vital role by convening stakeholders from the legal, clinical, and technology communities to develop a consensus framework for shared liability and indemnification. Such a framework should be fair to all parties and, critically, should not place an undue burden on frontline clinicians that would chill their willingness to use beneficial tools.

Privacy, Security, and Data Governance. AI models, particularly those hosted in cloud environments, require robust data governance to protect patient privacy. As a PACE organization, we are the fiduciaries of our participants' most sensitive health information. We need clear, enforceable standards for data use agreements with AI vendors, particularly concerning the secondary use of data for model training and the prevention of re-identification. HHS should consider issuing updated guidance under HIPAA that specifically addresses the unique privacy considerations of AI in healthcare.

Question 4: Promising AI Evaluation Methods for Non-Medical Devices

For AI tools that are not regulated as medical devices, evaluation must extend beyond traditional accuracy metrics to encompass the full context of clinical use.

Workflow and Human-Centered Evaluation. The most promising evaluation methods assess how an AI tool integrates into the clinical workflow and its impact on the humans who use it. Key questions include: Does the tool save time or add steps? Does it reduce cognitive load and decision fatigue? Does it enhance clinician confidence, or does it create confusion and alert fatigue? Rigorous usability testing and qualitative research with end-users should be a standard component of any AI evaluation.

Post-Deployment Monitoring and Surveillance. An AI model that performs well in a controlled testing environment may degrade over time in the real world as patient populations shift and care patterns evolve, This is a phenomenon known as "model drift." Continuous post-deployment monitoring is essential to ensure that AI tools remain accurate, fair, and equitable over time. This is particularly critical for our diverse, multilingual participant population.

HHS can support these processes through several mechanisms. Grants and cooperative agreements could fund the development of standardized, open-source testing methodologies for workflow integration and human factors. HHS could also establish a national repository of best practices for post-deployment AI surveillance, potentially housed within an existing agency like AHRQ or ONC. Prize competitions could incentivize the development of innovative tools for detecting algorithmic bias and model drift in real-world clinical settings.

Question 5: Supporting Private Sector Activities

HHS can most effectively support private sector activities by acting as a convener, a standard-setter, and a catalyst for collaboration.

Convene an AI for Geriatrics Center of Excellence. We strongly recommend that HHS facilitate the creation of a public-private partnership focused specifically on AI for the elderly population. Such a center could bring together PACE organizations, academic medical centers, AI developers, and patient advocates to address the unique challenges of this underserved market. Its activities could include creating and validating benchmark datasets representative of the geriatric population, developing testing and certification standards for AI tools intended for use in geriatric care, and disseminating best practices for ethical and effective implementation.

Recognize and Promote Industry-Led Standards. HHS should actively monitor and, where appropriate, endorse industry-led efforts to develop standards for AI safety, transparency, and accountability. By signaling which voluntary standards it considers credible, HHS can help healthcare organizations make informed procurement decisions and encourage AI developers to invest in responsible practices.

Question 6: AI Performance and Potential

In our experience, the AI tools that have delivered the most immediate and measurable value are those focused on operational efficiency and care coordination. For a PACE organization, logistics are paramount. Tools that optimize transportation routing, predict appointment no-shows, or automate administrative tasks like prior authorizations can yield significant time and cost savings, freeing up staff to focus on direct participant care.

However, the greatest potential for transformative impact lies in predictive analytics for preventive care. For a frail senior population, the ability to accurately predict the risk of an adverse event, such as a fall, a urinary tract infection, a hospitalization for heart failure exacerbation, or a decline in functional status, would be profoundly valuable. Such a tool would enable our interdisciplinary care teams to intervene proactively, adjusting care plans, increasing home visits, or initiating therapy before a crisis occurs. This aligns perfectly with the PACE model's emphasis on keeping participants healthy and independent in their homes.

Areas where AI has fallen short tend to involve tools that were not designed with the geriatric population in mind. Generic clinical decision support tools trained on younger, healthier populations often generate irrelevant or even harmful recommendations for our participants with multiple chronic conditions and complex medication regimens.

Question 7: Organizational Decision-Makers and Administrative Hurdles

Within a healthcare organization like CEI, the decision to adopt a significant new technology like AI is inherently collaborative, involving multiple stakeholders.

Role	Primary Concerns
Chief Medical Officer / Medical Director	Clinical efficacy, patient safety, impact on quality of care, clinician workflow
Chief Operating Officer	Operational efficiency, implementation logistics, staff training, scalability
Chief Information Officer	Technical feasibility, integration with existing systems, data security, vendor management
Chief Financial Officer	Cost, return on investment, budget allocation, reimbursement implications
Board of Directors	Strategic alignment, fiduciary responsibility, risk management

The primary administrative hurdle to AI adoption is the business case. Without a clear, demonstrable link between an AI investment and either a new revenue stream, a reduction in costs, or an improvement in quality metrics that directly affect payment or accreditation, it is difficult to secure the necessary budget and organizational commitment. HHS can help overcome this hurdle by funding research that quantifies the ROI of AI in value-based care settings and by creating payment incentives that reward AI-driven quality improvements.

Question 8: Interoperability to Widen Market Opportunities

Enhanced interoperability would be the single most powerful catalyst for accelerating AI development and widening market opportunities. The current fragmented data landscape creates a significant barrier to entry for AI developers, who must navigate a patchwork of proprietary systems and inconsistent data formats.

If HHS were to successfully mandate and enforce true interoperability, it would enable the creation of comprehensive, longitudinal health records that could fuel a new generation of AI tools. For the geriatric care market specifically, we would highlight the following priorities:

Data Type / Standard	Impact on AI Development
Standardized ADL/IADL Assessments	Would enable AI models to predict functional decline and tailor care plans to maintain independence.
Social Determinants of Health (SDOH) Data	Standardized SDOH data (e.g., using Gravity Project standards) would allow AI to identify and address non-medical factors that profoundly impact senior health.
Care Plan Interoperability	Standardized, shareable care plans would enable AI to support care coordination across the entire care team, including external providers.
Medication Reconciliation Data	Comprehensive, real-time medication data would enable AI to identify polypharmacy risks and drug interactions, a critical issue for seniors.

Question 9: Patient and Caregiver Perspectives

Our participants and their caregivers have expressed both hopes and concerns regarding the adoption of AI in their care.

Challenges They Wish AI to Address. Participants and caregivers are most interested in AI tools that simplify the practical burdens of managing complex care. This includes easier communication with their care team, more convenient scheduling of appointments and transportation, personalized reminders for medications and health activities, and accessible, understandable health education. For caregivers, tools that provide peace of mind, such as remote monitoring or alerts for potential health changes, are highly valued.

Concerns About AI Adoption. The primary concern expressed by our participants and their families is the potential for technology to depersonalize care. They fear that an over-reliance on AI could diminish the human connection, compassion, and trust that define their relationship with our staff. For many of our participants, the social interaction at our PACE centers is as important to their well-being as the medical care they receive. Any AI strategy must be implemented in a way that enhances, rather than erodes, these vital human relationships. The "Augmented Intelligence" framing directly addresses this concern by making clear that technology is a tool to support, not supplant, human caregivers.

Question 10: AI Research Priorities

HHS should prioritize research that addresses the specific needs of vulnerable populations and generates the evidence base needed to support broader AI adoption.

AI for Social Determinants of Health (SDOH). Research is urgently needed on how to effectively use AI to identify and address SDOH factors, such as social isolation, food insecurity, housing instability, and transportation barriers, that have an outsized impact on the health of seniors. This includes developing models that can integrate SDOH data with clinical data to generate actionable insights for care teams.

Longitudinal Studies on AI Impact in Value-Based Care. HHS should fund rigorous, long-term studies to measure the costs, benefits, and clinical impact of AI tools within integrated, value-based care models like PACE. Such studies should examine not only clinical outcomes but also participant and caregiver experience, staff satisfaction, and total cost of care. This evidence is essential to justify broader adoption and to inform the design of appropriate reimbursement policies.

Ethical AI for Vulnerable Populations. Research is critically needed to develop robust methods for detecting and mitigating bias in AI models when applied to vulnerable populations, including frail elders, individuals with cognitive impairment, and those from historically marginalized communities. This research should inform the development of standards and best practices for ensuring that AI in healthcare is fair, equitable, and trustworthy.

Regarding published findings (Question 10a), while the literature on AI in healthcare is growing rapidly, there is a notable gap in high-quality evidence specifically focused on the geriatric population and on integrated care models like PACE. Most published studies focus on narrow clinical applications in acute care settings. HHS-funded research could help fill this gap.

Regarding the literature on costs and benefits (Question 10b), the existing literature tends to focus on the potential cost savings from AI, often with optimistic assumptions. There is less rigorous analysis of the full economic picture, including implementation costs, ongoing maintenance, and the distribution of benefits and burdens across different stakeholders. A more nuanced, comprehensive approach to economic evaluation is needed.

Conclusion

The Center for Elders' Independence is deeply committed to harnessing the power of Augmented Intelligence to improve the lives of the frail seniors we serve. We believe that the PACE model, with its integrated structure and value-based incentives, represents an ideal environment for the responsible development and deployment of AI in clinical care.

We thank the Department of Health and Human Services for its leadership in this critical area and for the opportunity to contribute our perspective. We stand ready to partner with HHS, whether through pilot programs, research collaborations, or policy development, to build a future where technology empowers compassionate, equitable, and effective care for all Americans.

Respectfully submitted,

Amarjit Dhillon
Chief Information Officer
Center for Elders' Independence
510 17th Street, Oakland, CA 94612
adhillon@cei.elders.org

This response is submitted in accordance with the instructions provided in the Federal Register notice (90 FR 60108) and is identified as a response to the "HHS Health Sector AI RFI."